NCCP 00421 · VERSION 6

Your treatment, explained

Locally advanced rectal cancer · NCCP 00421 v6 · Document revised 4 November 2026

Your personal plan: this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

Continuous fluorouracil + radiotherapy

7-day cycle | Source reconciled; independent clinical, pharmacy and patient/PPI review pending

ACT ON SYMPTOMS, NOT THE CALENDAR

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Contact oncology immediately for fever, shivering, sudden illness, severe diarrhoea, inability to drink, very little urine or unusual bleeding. New severe chest pain, collapse or major breathing difficulty needs emergency help.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Open your CUH contact / treatment card

Sources: nccp

YOUR COMPLETE REFERENCE

Read what you need, when you need it.

Start with the overview, explore the cycle, or go straight to a question. Urgent advice is always available.

- How each drug works
- Your treatment plan
- Your 7-day timeline
- Baseline and cycle-specific checks
- Whole treatment, scans and checks
- Your port and treatment pump
- Tablets, visits and practical planning
- Your treatment aim and duration
- Tests and safety
- Recognise symptoms and act
- Daily life and support
- Radiotherapy and recovery
- Your home infusion
- Supportive medicines
- Understanding the evidence
- Questions to ask
- Sources

UNDERSTAND YOUR TREATMENT

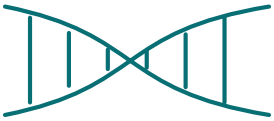
Your medicines and what they do

Continuous fluorouracil helps radiotherapy treat locally advanced rectal cancer. Your team explains how this fits with surgery and other treatment.

CHEMOTHERAPY

Fluorouracil

5-FU



Disrupts cell building blocks

Fluorouracil interferes with DNA and RNA, the instructions cells need to grow. The same drug has different schedules; follow the prescription for this regimen.

Drug enters cell → DNA / RNA disrupted → Growth slows

What to watch for

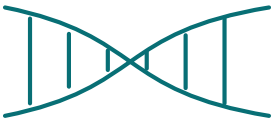
Report diarrhoea, mouth sores, painful hands or feet and unexpected severe early symptoms. Chest pain needs urgent assessment even after a normal DPD test.

Sources: nccp

LOCAL RADIATION TREATMENT

Pelvic radiotherapy

External beam treatment



Targets the pelvic treatment area

Radiation is directed at the planned area. Fluorouracil helps sensitise cancer cells to it. Nearby bowel, bladder and skin may also become irritated. Effects can build over the weeks and may continue after the final session.

Planned radiation field → Damage to cancer-cell DNA → Cells lose the ability to multiply

What to watch for

Tell the radiotherapy team about diarrhoea, pain, skin soreness or urinary changes. Discuss possible longer-term bowel, bladder, sexual and fertility effects before treatment.

Sources: nccp

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Explore your existing anatomy and side-effect map

REGIMEN-SPECIFIC SIDE EFFECTS

Your anatomical side-effect map

A picture of side effects

Match each symptom box to its numbered body location. Tell your team about new symptoms.

1 Mouth / eating

Fluorouracil

Report painful ulcers or difficulty eating and drinking.

3 Bowel / hydration

Fluorouracil + pelvic radiotherapy

Report diarrhoea, increased stoma output, severe pain, blood or dehydration.

5 Blood / infection

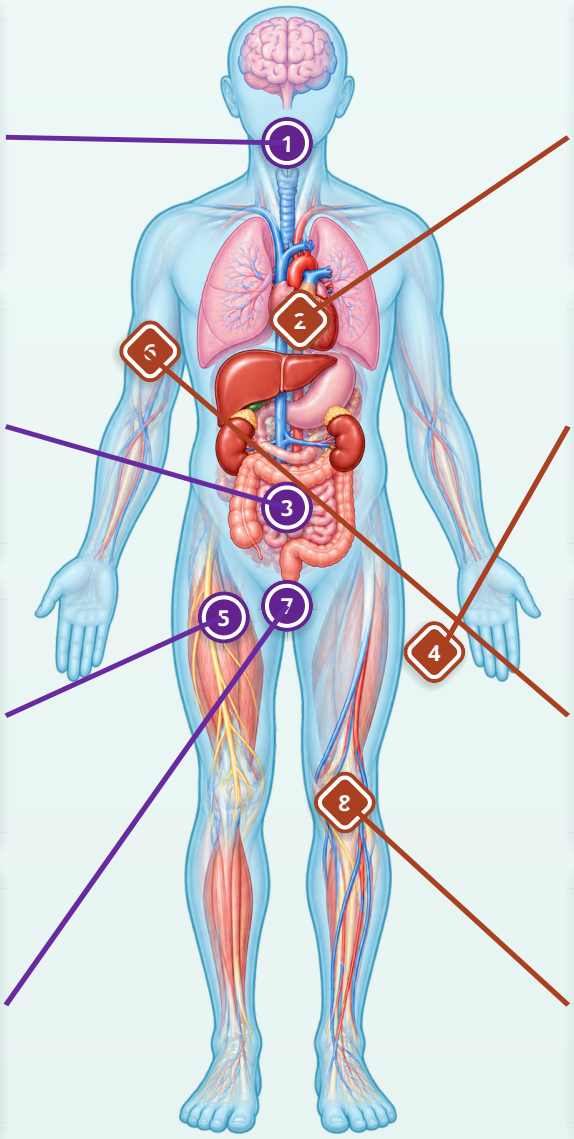
Chemotherapy

Fever, shivers or sudden illness need immediate oncology advice.

7 Pelvic skin / bladder

Radiotherapy

Report skin pain, urinary symptoms or bleeding; effects can continue after treatment.



2 Chest / heart

Fluorouracil

New chest pain needs urgent assessment, including during an infusion.

4 Hands / feet

Fluorouracil

Report painful redness, cracking or peeling that affects daily activity.

6 Pump / line

Fluorouracil delivery

Report leakage, line pain, swelling, disconnection or a stopped pump immediately.

8 Recovery / daily life

Chemoradiotherapy

Tell the team if tiredness, pain or poor intake limits normal activities.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

THE BIGGER TREATMENT PLAN

Your treatment plan

LOCALLY ADVANCED RECTAL CANCER

Prescribed treatment course



Fluorouracil runs continuously through a pump, seven days a week, during a six-week radiotherapy course. The pump is changed every seven days. Radiotherapy has its own appointment calendar.

SEPARATE APPOINTMENTS

Radiotherapy plan



The radiation team confirms the field, fractions, appointments and aftercare; pump days do not define radiation attendance.

The treating team confirms the indication, schedule and duration. Changes are recorded on your own prescription.

Sources: nccp

BASELINE → TREATMENT → FOLLOW-UP

Tests and checks

Required safety blood tests are separate from the individually prescribed scan or tumour-marker plan.

Check	Before starting	Treatment visits	Between visits	Review	After treatment
Blood count / kidneys / liver	Baseline tests and DPD status	Weekly throughout treatment	Extra tests if unwell or dehydrated	Results guide any delay or change	Follow unresolved abnormalities
Symptoms and medicines	Review medicines, cardiac history and usual function	Review bowel, mouth, skin and infection symptoms	Use urgent contact plan	Confirm recovery and next dose	Agree recovery care

Check	Before starting	Treatment visits	Between visits	Review	After treatment
Scans / tumour markers	Confirm disease assessment	According to individual plan	New symptoms may bring review forward	Consultant-directed response assessment	Individual follow-up
Radiotherapy / pump	Radiation planning; line and pump training	Weekly bloods and pump changes; separate radiation visits	Report bowel, bladder, skin or line problems	Assess nutrition and hydration	Radiation/colorectal team decides response assessment and next steps

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00421 v6 official regimen

THE WHOLE TREATMENT JOURNEY

Continuous fluorouracil + radiotherapy: treatment and checks

Locally advanced rectal cancer

Fluorouracil runs continuously through a pump, seven days a week, during a six-week radiotherapy course. The pump is changed every seven days. Radiotherapy has its own appointment calendar.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

On a small screen, swipe the map sideways. The checks below give the same timing in words.

Treatment, tests and reviews across the course

Care / check	Before starting	Treatment visits	Between visits	Review	After treatment
Blood count / kidneys / liver NCCP 00421	Baseline tests and DPD status	Weekly throughout treatment	Extra tests if unwell or dehydrated	Results guide any delay or change	Follow unresolved abnormalities
Symptoms and medicines NCCP 00421	Review medicines, cardiac history and usual function	Review bowel, mouth, skin and infection symptoms	Use urgent contact plan	Confirm recovery and next dose	Agree recovery care
Scans / tumour markers NCCP 00421	Confirm disease assessment	According to individual plan	New symptoms may bring review forward	Consultant-directed response assessment	Individual follow-up
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What each check is for

1

Confirm your personal plan

Before starting

Continuous fluorouracil helps radiotherapy treat locally advanced rectal cancer. Your team explains how this fits with surgery and other treatment.

Ask / act: Agree medicines, treatment days, duration and monitoring.

Sources: NCCP 00421

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-04 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.32

Day 1 → Day 7: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Fluorouracil runs continuously through a pump, seven days a week, during a six-week radiotherapy course. The pump is changed every seven days. Radiotherapy has its own appointment calendar.

Day of cycle	1	2	3	4	5	6	7
Treatment pattern	Fluorouracil runs continuously through a pump, seven days a week, during a six-week radiotherapy course. The pump is changed every seven days. Radiotherapy has its own appointment calendar.						
Symptoms and infection	Report on any day; timing varies						

The written plan comes first: Fluorouracil runs continuously through a pump, seven days a week, during a six-week radiotherapy course. The pump is changed every seven days. Radiotherapy has its own appointment calendar.

Symptoms can continue: Contact oncology for fever, sudden illness, diarrhoea, mouth soreness or inability to drink, during treatment or the break.

Why these bands stay broad: Bars show planned treatment and possible symptom patterns, not measured risk. Side effects can overlap and persist. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00421 v6 official regimen ↗

There are no guaranteed "safe days". Act on fever, sudden illness or a new serious symptom whenever it occurs. Do not wait for the next appointment.

Read the whole cycle as text

Day 1: Weekly review and pump change

Check blood tests, line care and the next pump appointment. Side effects can occur on any day; the calendar does not predict when you will feel well. Call the oncology team if unwell. Do not change pump settings or disconnect it yourself unless specifically trained and instructed.

Day 2–7: Continuous treatment, including weekends

Keep the pump connected as instructed; radiation appointments follow their own calendar. Side effects can occur on any day; the calendar does not predict when you will feel well. Call the oncology team if unwell. Do not change pump settings or disconnect it yourself unless specifically trained and instructed.

YOUR TREATMENT, STEP BY STEP

Your port and treatment pump

From preparing the line to coming home after disconnection. Your team arranges the appointments and shows you what to do.



1 · Before treatment

Choose and prepare the line

The team explains whether a port or PICC suits your treatment. You receive instructions for the insertion appointment.



2 · Insertion and recovery

A separate appointment may be needed

For a port, numbing medicine is used and a small device is placed under the skin. Ask about sedation, transport and wound care.



3 · Day 1

The nurse connects your pump

After the day-ward medicines, the nurse checks and connects the fluorouracil pump. Ask for a demonstration before leaving.

4 · At home

Continuous infusion throughout the week

Fluorouracil runs continuously, seven days a week, during the six-week radiotherapy course. Keep the pump and dressing protected as taught.

5 · Every seven days

Professional pump change

The team changes the pump weekly and checks the line. Attend the separate radiotherapy schedule. Do not disconnect or change the pump yourself.

6 · Throughout the course

Ongoing line care

The access device stays in place as planned. Keep the written line-care and urgent-contact instructions; the team confirms completion and disconnection.

Port or PICC: what is the difference?

A port is a small chamber beneath the skin, usually on the chest, attached to a tube ending in a large vein. A special needle goes through the skin into the port when it is used. A PICC enters a vein in the upper arm; a short section remains outside under a dressing. Both can deliver a home infusion. Your team helps choose the device and teaches its care.

Before insertion and during recovery

Tell the team about allergies, blood thinners, infections and any other cancer medicines. Follow the instructions about eating, drinking and transport; do not stop medicines yourself. Port placement uses local anaesthetic, sometimes with sedation or general anaesthetic. The team explains the procedure and risks, including infection, bleeding, clots and insertion complications. Ask how to manage discomfort, protect the wound and resume activity. Report increasing pain, redness, swelling, discharge or fever.

Connection: what the nurse does

The nurse checks the line and treatment, secures the connection and dressing, and starts the pump. Before leaving, you should have the disconnection location and time, contact numbers including out of hours, and the written spill instructions. Ask to be shown what normal pump progress looks like. You do not need to calculate a rate.

Carrying it, sleeping and washing

Use the supplied carrier and the position recommended for your pump. Keep tubing free from pulling and kinks. Ask how to place the pump safely for sleep and how to protect the line and dressing when washing. Do not immerse it or swim. Avoid direct heat and cold. An elastomeric pump has a balloon that shrinks slowly and may have no alarm; an electronic pump has different instructions. Follow the instructions for the device you actually receive.

Disconnection is not port removal

Keep the appointment even if the pump looks empty, unless the team changes the plan. Staff check the pump and line and arrange safe disposal. After flushing and other line care as required, the port needle may be removed and a small dressing applied. The implanted port stays in place until a separate removal procedure is arranged. A PICC remains in place and needs scheduled dressing and flushing care.

Leakage, disconnection or unexpected pump progress

Call the team immediately if the pump leaks, is damaged, finishes unexpectedly early, appears not to run, or the line disconnects. Follow the emergency or spill steps you have been specifically taught, including use of a clamp if instructed. Do not reconnect, change settings or disconnect the pump yourself. Keep other people away from leaked medicine. If medicine touches skin, rinse it off and wash with soap and water, then seek advice.

Pain, swelling or feeling unwell

Contact oncology immediately for pain or swelling around the line, a wet/loose dressing, redness, fever, shivering or suddenly feeling unwell. A painful swollen arm or neck needs urgent advice. Severe chest pain, major breathing difficulty or collapse: call 112 / 999. Do not wait for pump disconnection or the next clinic visit.

The drawings show general steps, not instructions for inserting or accessing a line. Your hospital confirms its own arrangements and the care instructions for your device.

Open the complete port and pump guide · **Printable port and pump guide**

Sources: eviQ: port · eviQ: PICC · eviQ: pump. Reviewed against these sources 16 October 2026; local nursing and patient review pending.

PRACTICAL TREATMENT GUIDE

Practical treatment planning

Agree the arrangements with your oncology team.



STEP 1

Before treatment

Confirm the treatment aim, duration, DPD result, blood tests, prescription and contact numbers.



STEP 2

Visits and access

Fluorouracil runs continuously through a pump, seven days a week, during a six-week radiotherapy course. The pump is changed every seven days. Radiotherapy has its own appointment calendar. Confirm line access, pump changes and disconnection arrangements.



STEP 3

If treatment is delayed

Ask for revised written treatment and support-medicine instructions. Do not make up missed doses or carry an old plan into a new phase.



STEP 4

Getting help

Keep the wallet card and medicine list available. Contact the team promptly for infection, dehydration, chest symptoms or significant toxicity.

Open practical treatment guide · Printable practical guide

Sources: NCCP 00421 v6 official regimen. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.

MAKE THE PLAN PRACTICAL

Support around the medicines

1 Supportive medicines

Use the prescribed anti-sickness and diarrhoea plan; check the medicine, timing and rescue duration. Do not duplicate steroids or anti-sickness medicines.

2 Mouth, bowel and skin care

Report soreness and diarrhoea early. Use gentle mouth and skin care and ask for help with food or fluids.

3 Medicine checks

Bring all prescription medicines, supplements and non-prescription products. Warfarin and phenytoin may need closer monitoring; brivudine/sorivudine interactions require pharmacy review.

4 Pump and line support

Confirm the pump-care instructions and a working contact number. Do not adjust, reconnect or disconnect the pump unless specifically trained and instructed.

Sources: nccp

TREATMENT REFERENCE · 01

Your treatment aim and duration

Why this treatment?

Continuous fluorouracil helps radiotherapy treat locally advanced rectal cancer. Your team explains how this fits with surgery and other treatment.

Sources: nccp

Your course

Fluorouracil runs continuously through a pump, seven days a week, during a six-week radiotherapy course. The pump is changed every seven days. Radiotherapy has its own appointment calendar.

Sources: nccp

Choices

Discuss the expected benefit, burdens, alternatives and how treatment fits your priorities. You can ask for more time or another explanation before deciding.

Sources: nccp

TREATMENT REFERENCE · 02

Tests and safety

DPD before fluorouracil

Low DPD activity can cause severe toxicity. Testing helps guide the plan but cannot prevent every reaction. Unexpected severe early symptoms need urgent advice.

Sources: nccp

Blood tests

Weekly throughout treatment: blood count, renal and liver profile; baseline DPD. Scans/ markers follow the consultant plan.

Sources: nccp

Medicines and interactions

Bring all prescription medicines, supplements and non-prescription products. Warfarin and phenytoin may need closer monitoring; brivudine/sorivudine interactions require pharmacy review.

Sources: nccp

TREATMENT REFERENCE · 03

Recognise symptoms and act

Mouth / eating

Report painful ulcers or difficulty eating and drinking.

Sources: nccp

Chest / heart

New chest pain needs urgent assessment, including during an infusion.

Sources: nccp

Bowel / hydration

Report diarrhoea, increased stoma output, severe pain, blood or dehydration.

Sources: nccp

Hands / feet

Report painful redness, cracking or peeling that affects daily activity.

Sources: nccp

Blood / infection

Fever, shivers or sudden illness need immediate oncology advice.

Sources: nccp

Pump / line

Report leakage, line pain, swelling, disconnection or a stopped pump immediately.

Sources: nccp

Pelvic skin / bladder

Report skin pain, urinary symptoms or bleeding; effects can continue after treatment.

Sources: nccp

Recovery / daily life

Tell the team if tiredness, pain or poor intake limits normal activities.

Sources: nccp

TREATMENT REFERENCE · 04

Daily life and support

Food and fluids

Aim for tolerable meals and fluids. Report weight loss or poor intake; ask for dietetic support.

Sources: nccp

Activity and rest

Adjust activity and rest to your energy. Report fatigue that limits normal tasks.

Sources: nccp

Fertility and sexual health

Ask about fertility preservation before treatment and the contraception and breastfeeding plan for all medicines. Report a possible pregnancy promptly.

Sources: nccp

Support for you and your family

Ask about counselling, psycho-oncology, practical support, transport and financial advice. Bring someone to discussions if helpful.

Sources: nccp

TREATMENT REFERENCE · 05

Radiotherapy and recovery

Separate appointments

Follow the radiation team’s preparation and appointment calendar. Seven-day infusion does not mean seven radiation visits per week.

Sources: nccp

Pump throughout the week

The fluorouracil pump runs continuously, including weekends, and is changed every seven days. A radiation interruption requires an explicit chemotherapy plan.

Sources: nccp

After treatment

Bowel, bladder, skin and fatigue effects may persist or develop later. Agree follow-up and the next colorectal treatment decision.

Sources: nccp

TREATMENT REFERENCE · 06

Your home infusion

Before leaving

Check the exact finish or change time, disconnection appointment, line care and 24-hour contact route.

Sources: nccp

If something goes wrong

Report leakage, swelling, line pain, disconnection, an alarm or unexpected pump speed. Follow the supplied spill instructions; do not handle leaked medicine with bare hands.

Sources: nccp

Changes to treatment

Do not restart, reconnect or change the pump unless specifically trained and instructed. The team updates the prescription and pump plan.

Sources: nccp

Understanding the evidence

Ask which evidence matches your treatment aim.

The source supports continuous fluorouracil with radiotherapy for locally advanced rectal cancer. Radiation technique, later surgery and other systemic therapy affect outcomes; no survival estimate from a different schedule is applied.

Sources: nccp

PREPARE FOR THE CONVERSATION

Questions worth asking

Which treatment plan applies to me?

Fluorouracil runs continuously through a pump, seven days a week, during a six-week radiotherapy course. The pump is changed every seven days. Radiotherapy has its own appointment calendar. Your team confirms any modification.

Do more side effects mean treatment is working?

No. Side effects do not reliably measure response. Report symptoms and follow the agreed review plan.

Who do I contact overnight?

Ask the team to replace example numbers with working daytime, out-of-hours and backup contact routes before leaving.

> **Try explaining the plan back:** Can you explain which medicines you are receiving, the schedule, the contact plan and the symptoms that need prompt help?

SOURCE TRANSPARENCY

Where this information comes from

Regimen schedule: NCCP 00421 v6. Medicine explanations and safety information: sources below. The timeline is an educational illustration, not a measured risk curve or an individual prediction.

- NCCP 00421 v6 official regimen
- NCCP NCIS antiemetic guidance V8

SACTCheck · Developed by Dr Paul O'Brien · Sources checked 2026-11-04.
This information supports your treatment discussion. Your oncology team confirms your personal treatment and urgent-care plan.

Written source addresses

https://healthservice.hse.ie/documents/6460/421_5-Fluorouracil_and_RT.pdf

https://www.eviq.org.au/p/507

https://www.eviq.org.au/p/543

https://www.eviq.org.au/p/502

https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf